CITY CARE HEART INSTITUTE

Department of Cardiology | Sample document with a fictional patient, for demonstration only

DISCHARGE SUMMARY

Patient Name: Mr. Ramesh Kumar	**Age / Sex:** 58 Y / M	**UHID:** CCH/26/104578
DOA: 02-Oct-2026	**DOD:** 07-Oct-2026	**IP No.:** IP-26-08812
Consultant: Dr. A. Sharma, MD, DM (Cardiology)		**Ward:** CCU / Cardiac Ward

FINAL DIAGNOSIS

- Acute Inferior Wall MI (STEMI), Killip Class I
- CAD: SVD (100% occlusion of mid RCA)
- T2DM (uncontrolled), HTN

PROCEDURE DONE

CAG followed by Primary PTCA + 1 DES to mid RCA via right radial approach on 02-Oct-2026. TIMI III flow achieved. No peri-procedural complications.

PRESENTING COMPLAINTS

C/o retrosternal chest pain radiating to left arm with diaphoresis x 3 hrs. H/o T2DM x 8 yrs, HTN x 5 yrs. Chronic smoker (10 pack-years). No h/o drug allergy.

INVESTIGATIONS

Test	Result	Test	Result
ECG (on admission)	ST elevation in II, III, aVF	Trop-I	12.4 ng/mL (raised)
2D Echo	RWMA inferior wall, LVEF 45%	HbA1c	8.2 %
S. Creatinine	1.0 mg/dL	LDL-C	142 mg/dL
Hb	13.1 g/dL	BP at discharge	128/80 mmHg

COURSE IN HOSPITAL

Pt. was admitted in CCU, loaded with DAPT and statin and taken up for primary PCI. Post-procedure course was uneventful. Pt. was haemodynamically stable, ambulated on POD-2 and is being discharged in stable condition.

MEDICATIONS ON DISCHARGE

#	Drug	Dose	Frequency	Duration
1	Tab. Ecosprin (Aspirin)	75 mg	0-1-0 (after lunch)	Lifelong
2	Tab. Brilinta (Ticagrelor)	90 mg	1-0-1 (BD)	12 months, do not stop
3	Tab. Atorva (Atorvastatin)	40 mg	0-0-1 (HS)	To continue
4	Tab. Met-XL (Metoprolol succinate)	25 mg	1-0-0 (OD)	To continue
5	Tab. Cardace (Ramipril)	2.5 mg	0-0-1 (OD)	To continue
6	Tab. Glycomet (Metformin)	500 mg	1-0-1 (BD, after meals)	To continue
7	Tab. Pan (Pantoprazole)	40 mg	1-0-0 (BBF)	30 days
8	Tab. Sorbitrate (Isosorbide dinitrate)	5 mg	S/L SOS for chest pain	As needed

ADVICE ON DISCHARGE

- Low salt, low fat, diabetic diet. Fluid as per thirst.
- Strictly no smoking / tobacco / alcohol.
- Avoid lifting heavy weights and strenuous exertion for 4 weeks. Start walking 10 min twice daily and increase gradually.
- Keep the right wrist puncture site clean and dry for 3 days.
- Do NOT stop antiplatelets (Ecosprin, Brilinta) without the cardiologist's advice.
- Monitor FBS / PPBS at home twice a week and maintain a chart.

FOLLOW-UP

- Review in Cardiology OPD with Dr. A. Sharma on **14-Oct-2026 at 10:30 AM** with a fresh ECG.
- Repeat Lipid Profile and HbA1c after 6 weeks.
- Repeat 2D Echo after 3 months.

WHEN TO REPORT TO EMERGENCY

Chest pain not relieved by S/L Sorbitrate within 5 minutes, breathlessness, giddiness / syncope, palpitations, black stools or any bleeding, swelling or bleeding at the puncture site. Emergency helpline: 1800-000-0000 (24x7).

Prepared by: Dr. N. Verma (Resident)

Dr. A. Sharma
MD, DM (Cardiology)

This is a fictional sample created for a software demonstration. It does not describe a real patient or hospital.